

Obstetrics
O-05 Severe Pre-Eclampsia / Eclampsia

Revised 3/1/2000

PRIORITIES:

- ABCs
- Identify the signs of shock;
- Determine stage (trimester) of pregnancy (if pregnant);
- Determine the degree of physiologic distress, estimate amount of blood loss;
- Assure an advanced life support response.

Severe Pre-Eclampsia/Eclampsia

Third trimester pregnancy with hypertension (BP systolic >160, diastolic >110), mental status changes, visual disturbances, peripheral edema (pre-eclampsia), seizures and/or coma (eclampsia).

1. Ensure a patent airway (suction as necessary);
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY - Begin oxygen at 6 liters/minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respiration as needed. DO NOT withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD;
4. Attempt to maintain a quiet environment;
5. Monitor vital signs frequently;
6. Assist advanced life support personnel with patient packaging and movement to ambulance;
7. Treat seizures, coma and hypertension per COMA/SEIZURES/HYPERTENSIVE EMERGENCY guidelines.